



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : عايده قرني الشرييني الهويشي

العنوان : 0

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 26803110102601 رقم التليفون : 01140366671

العمر (تاريخ الميلاد) : 11-03-1968 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإحصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: عايدہ قرني الشريني الهويشي National ID: 26803110102601	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روستاف
9-29



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السنة المصرية
٢٠٢٦

09/29/2026

MDT FORM

Name:	Ayda Korne Elsherbene Elhweshe	MRN:	12160
Age:	59 Y	ID:	26803110102601
Menopausal Sta		Phone:*	01240366671
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve fh
Gender		Reason Of MDT:	
CO Morbidity:	m.free	ECOG Performance Status	0 0 1 0 2 0 3 0 4
Surgical History:*	nad		

Clinical Data:

BX: rt idc g3
ER: 4/8 **PR:** 3/8 **HER2:** +3 **KI67:** 25%

History:

Sonomammography:

acr b
rt uoq mass at 10 oclock zone a 2cm from nipple 3mm from overlying skin
2.2*1.6 cm no pathological axillary ln birads 4

Biomarker/Molecular

Mets Workup:

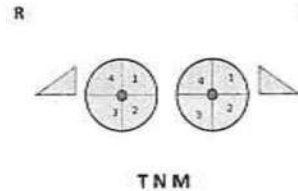
ct cap >> free
bone scan >> free

Recommendations:

for clipping nact then rt bcs & slnb

MDT Outcome & Plan

CESM + Bracketing by clips
Portacath Insertion
NACT --> TCHP x 6 cycles
Then rt bcs & slnb



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

ct2nomo

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د خالد عبد العزيز	د محمد خيرلوي		

جمهورية مصر العربية
بطاقة تحقيق الشخصية

عايدة
قرنى الشربيني الهويشى
٥٦ التمييز حتى الاسمرات
المقطم - القاهرة

٢٦٨.٣١١.٠١.٢٦.٠١

LT2569804

٢٠٢٦/٠٥ ٢٦٨.٣١١.٠١.٢٦.٠١

بدون عمل

أنثى
متزوجة
اممن محمد عبدالله عبدالوهاب

البطاقة سارية حتى ٢٠٣٣/٠٥/٠٩

١

دms

معاينة

٢٦٨

Patient Name	عايدة قرني الشربيني الهويشي	Case No.	263906
Sex/Age	Female 58 Y	Patient No.	20205
Registered Date	01/6/2026 01:20:40 PM	Referred By Dr.	Interventional Radiology
Validation Date	25/6/2026 01:12:49 PM	Branch	Gustave Roussy
Reporting Date	28/6/2026 08:47:19 AM	Pathology No.	1717/26

Pathology Report

Clinical Data: Right breast UOQ mass (BIRADS 4c).

Nature of Specimen: Tru-cut biopsy, collected on 01/06/2026, at 10:10 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey-white tissue cores, ranging in size between 1.0 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 3 (differentiation 3, pleomorphism 3, mitosis 2).
- **Ductal carcinoma in situ:** Focal intermediate grade DCIS (Solid pattern with comedo type necrosis).
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Results:

*** IPT (All controls showed appropriate reactivity):**

- ER: Weak nuclear reaction in 15% of tumor cells (score 4/8).
- PR: Weak nuclear reaction in 10% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.



Patient Name	عائده قرنى الشربيني الهويشى	Case No.	263906
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Reporting Date	28/6/2026 08:47:19 AM	Pathology No.	1717/26

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- Focal intermediate grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).

IPT:

- ER: Positive (score 4/8).
- PR: Positive (score 3/8).
- Her2: Positive (score +3).
- Ki67%: 25%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr Maha Guimei.
Dr. Walaa Ibrahim Ghonaim.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt



Name: Aida Korany Elsherbini .

Date: 5 May 2026

**DIGITIZED BILATERAL SOFT TISSUE MAMMOGRAPHY
& REAL TIME ULTRASONOGRAPHY FOR BOTH
BREASTS REVEALED:**

- ♦ Scattered fibroglandular dense mammary parenchyma of both breasts (ACR b).
- The right UOQ inner third shows an irregular shape lesion with speculation & suspicious micro-calcifications within the lesion.
- Normal skin thickness
- Intact nipple/ areolar complex.
- No mass lesion, distortion or asymmetry in left breast.

**REAL TIME ULTRASONOGRAPHY FOR BOTH BREASTS
REVEALED:**

Right UOQ irregular shaped hypoechoic lesion with speculated outline , It is surrounded by echogenic halo, seen at 10 o'clock inner zone a) about 2 cm from nipple and 3 mm from overlying skin . , It measures about 2.2x 1.6 cm .Intralesional vascularity was detected on color Doppler .

- No left breast solid or cystic lesions detected
- Normal retroareolar ducts
- Bilateral nonspecific axillary nodes with preserved fatty hilum .
- No pathological axillary nodes detected bilaterally .

OPINION:

Findings are suggestive of a highly suspicious right UOQ lesion with intralesional suspicious microcalcification (BI-RADS4c).

Normal left breast study (BI-RADS 1)

Recommendation:

- **Further management & biopsy is recommended.**

Best regards

Signature

Dr. Heba Ahmed MD.

Name : Aida Qarni El-Sherbini El-Huwaishi
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 20-09-2026 09:47
Medical No. : 465658
Ins.Company : تعاقد مستشفيات

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

58 year-old female patient, with history of right breast cancer and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No evidence of focal active or cold osseous lesion that would account for bone secondaries.
- Degenerative features are depicted at different levels of the spine.
- Osteoarthritic changes are noticed around big joints.

IMPRESSION:

- No definite osseous metastasis could be detected on scintigraphic basis.

**Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine**

Patient's Name: Aida Qarni Al-Sherbiny Al-Huwaishy
Date: 2026-08-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Suspected right breast lesion measuring 25 x 14 mm for sono-mammography.
- Diffuse mosaic attenuation of both lungs likely small airway disease for clinical and laboratory assessment.
- Few tiny bilateral scattered pulmonary nodule measuring 2-3 mm in diameter.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Left renal cortical cyst.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

Patient's Name: Aida Qarni Al-Sherbiny Al-Huwaishy
Date: 2026-08-12
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Hospital: Goustave Roussey International Egypt

- ❖ Few tiny bilateral scattered pulmonary nodules likely inflammatory for clinical and laboratory assessment.
- ❖ Otherwise no pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy

Fathh Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit



الجمعية الشرعية
مركز الفتح الإسلامي

أشعة مقطعية بالكمبيوتر - الوزن المنخفض
موجات صوتية - دوبلر ملون - رسم قلب بالجهد
رسم مخ - رسم عضلات - فحص ثدي - وحدة عيون

Name: Aida Korany Elsherbini.

Date: 5 May 2026

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- Best regards

Signature

Dr. Aida Ahmed MD.



Patient's Name:
Date:
Referred by:
Hospital:

Aida Qarni Al-Sherbiny Al-Huwaishy
2026-08-12
Prof.Dr. no doctor
Goustave Roussey International Egypt

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Opinion:



Patient's Name:
Date:
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Aida Qarni Al-Sherbiny Al-Huwaisby
2026-08-12
Prof.Dr. no doctor
Goustave Roussey International Egypt

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Dr.sherif Wagdy

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